



Public mental healthcare and economic vulnerability in Sri Lanka

Nadia Augustyniak¹

Social and Cultural Anthropology, Faculty of Social Sciences, University of Helsinki, PO 18 (Unioninkatu 35), 00014, Finland

ARTICLE INFO

Handling Editor: Dr E Mendenhall

Keywords:

Mental health
State services
Social inequality
Economic crisis
Social determinants of distress
Sri Lanka

ABSTRACT

Scholars and practitioners in Sri Lanka's mental health and psychosocial field have long highlighted the complex cultural, social and political dynamics of providing care to communities impacted by war and natural disaster and facing a fraught post-war context. One critical contribution of this work has been to offer a practice-based corrective to psychological conceptions of wellbeing that obscure its relational, economic, and political dimensions. In this article, I consider the importance of this insight in light of the 2022 debt crisis in Sri Lanka and against the backdrop of global mental health discourses that continue to elide the question of structural determinants of distress. Based on ethnographic research conducted between 2018 and 2020, I highlight the practice of government counselors and other mental health professionals in one district of Sri Lanka's Central Province. Their experiences suggest that despite expanded access to mental health services over the last 20 years, care is undermined by the structural realities of widespread economic precarity and inadequate social protections. This problematizes the global discourse of access to mental health care—which implies but often does not truly account for the social and economic bases of wellbeing—and underscores the fact that expansion of mental health services should go hand in hand with the expansion of social protections and economic support.

1. Introduction

Due to the economic difficulties faced by the Sri Lankan people today, there is a huge increase in mental problems and psychosocial problems. In that economic instability, it is more important for them to find a hired job for one meal a day than to seek remedies for their mental problems, and referring to counseling or related services is a loss (අහිමි වීමක්) of time and money according to them.

– Government Counseling Officer, Sri Lanka, 2023

In line with the heightened focus on mental health in national and global public health efforts, Sri Lanka has seen a significant expansion of its mental health services over the last twenty years (Minas et al., 2017). This has included the formulation of a national mental health policy, the decentralization of services, an increase in the number of providers, and the training of specialized mental health staff such as community psychiatric nurses. Yet, despite a dedicated cadre of mental health workers and increased access, care is undermined by the structural realities of widespread economic precarity and inadequate social protections.

Complementing the special issue's broader focus on post-war recovery in the context of continued political strife and economic crisis, in this article I draw on critical approaches to global mental health to consider more closely how social, economic and political realities shape public mental health care in Kandy District in Sri Lanka's Central Province.

The discussion is based on ethnographic research conducted between 2018 and 2020 and follow up conversations with practitioners in 2023 and 2024. Encompassing two nodes of state mental health care—district-wide counseling services and a divisional outpatient psychiatric clinic—the findings illustrate the tension between practitioners' recognition of their patients' and clients' psychosocial needs and the structural barriers to meaningfully addressing these in the context of medicalized mental health care and the underfunding of public services. This tension is captured in the opening quote by a government counselor reflecting on the impacts of the 2022 debt crisis on access to counseling. Implicitly, the statement points to the futility of such services in the absence of transformations in clients' everyday socio-economic, material struggles.

E-mail address: Nadia.Augustyniak@helsinki.fi.

<https://doi.org/10.1016/j.ssmmh.2024.100387>

Received 17 June 2024; Received in revised form 26 December 2024; Accepted 26 December 2024

Available online 27 December 2024

2666-5603/© 2024 The Author. Published by Elsevier Ltd. This is an open access article under the CC BY-NC-ND license (<http://creativecommons.org/licenses/by-nc-nd/4.0/>).

Across Sri Lanka's nine provinces, as well as among the rural, estate and urban sectors, there are significant disparities in rates of poverty, labor patterns and access to services.¹ Moreover, different parts of the country—and different communities—have been variously affected by the impacts of war and the 2004 Tsunami, shaping an uneven post-war landscape as well as somewhat divergent trajectories for the development of mental health and psychosocial services. The urban center of Kandy and surrounding divisions where the present research took place have been relatively well-resourced in terms of mental health and other medical and social services. Nevertheless, ground realities highlight the systemic challenge of political economic determinants of distress at a time when the economic crisis has deepened long-standing community vulnerabilities, doubling the national poverty rate and leading to widespread economic precarity.²

As I show below, the perspective of Sri Lankan practitioners as they grapple with their clients' psychosocial needs illustrates the broader point that the expansion of mental health services—whether in high or low and middle income countries—should be explicitly linked to the expansion of social protections.³ An unqualified policy focus on access to predominantly psychiatric and psychological services effectively elides the paramount importance of social and economic factors in people's lived experiences of distress—a central point highlighted in critical scholarship on global mental health (e.g., Kohrt and Mendenhall, 2016; Kottai and Ranganathan, 2020; Jain and Jadhav, 2009; Mills, 2014). Improved and equitable access to care is an undeniable need, yet such care must encompass robust psychosocial supports. In the following section I provide background on Sri Lanka's mental health sector and outline the critical perspectives that have shaped mental health and psychosocial services (MHPSS) in the country. In the remainder of the discussion, I focus on public counseling and psychiatric services to show how practitioners in Sri Lanka seek to provide mental health care in the context of widespread economic vulnerability among their patients and clients.

2. Background

Sri Lanka's landscape of mental health and psychosocial services has been shaped by the country's political history and local and international development practice. Affected by war (1983–2009) and two leftist insurrections (1971, 1987–89), parts of the country have had an NGO-based psychosocial and mental health support service structure in place since the 1980s (Somasundaram, 2014; Ganesan, 2011; The Asia Foundation, 2016). Counseling was beginning to be institutionalized in tandem with the expansion of clinical services in the state sector and subsequently, the field grew along different but often intertwined tracks in various parts of the country and in relation to a diversity of issues beyond the impacts of political violence and natural disaster. The 2004 Indian Ocean Tsunami marked a turning point in the mental health sector, highlighting the relative scarcity of psychiatric and psychological

services and spurring development efforts. The post-tsunami period coincided with the WHO's emphasis on expanding community-based mental health services in LMICs, which meant that there was an in-flow of funding for capacity building in the government health sector alongside developments in the already relatively robust NGO community-based and psychosocial services, which had grown since the 1990s.

In 2005 a National Mental Health Policy 2005–2015 was approved for the first time, outlining the need for decentralization and the expansion of community-based services. The effects are captured in growing numbers of personnel. In 2001, there were 38 psychiatrists practicing in the entire country, 25 of them in the Western Province (Minas et al., 2017). Care was highly centralized with the National Institute for Mental Health at Angoda and two additional hospitals in Colombo providing long-term stay facilities for patients and only some district hospitals offering acute in-patient care. There were also significant disparities in access across different parts of the country. As Minas et al. (2017) report, for example, in 2003 in the North and East there were only two psychiatrists and only two districts across the provinces had inpatient facilities.

One of the most important post-tsunami developments intended to address the shortage of personnel was the training of Medical Officers for Mental Health through a one-year diploma in psychiatry offered to general medical practitioners (Minas et al., 2017). By 2011 there were 110 MOMHs for a total of 276 MOH divisions in the country, which meant that there was now a provider per every 173, 000 population (Jenkins et al., 2012). Between 2005 and 2017, the number of consultant psychiatrists had also increased to 89 (Minas et al., 2017) and the mental health workforce had been expanded with new cadres of trained community psychiatric nurses and psychiatric social workers (Fernando et al., 2017). Although regional disparities persisted and the available personnel was considered inadequate to population need, these changes reflected tangible effects of decentralization policies and a marked increase in availability of services.

Reflecting these developments but also their uneven distribution, the urban area of Kandy where the present research was conducted is well-resourced when it comes to public community-based psychiatric services as well as psychological services. Kandy Teaching Hospital and Peradeniya Hospital, located only about a 20–30 min bus ride away from each other, offer comprehensive psychiatric services including the support of psychiatric social workers and nurses. Similarly, the government alcohol rehabilitation center at Mampitiya is located approximately 10 km outside the city. District-wide there are eight bi-weekly outpatient clinics as well as one long- and one medium-stay residential facility. At the time of writing, there were eight consultant psychiatrists and five Medical Officers for Mental Health in the district.

In the context of ongoing debates about the ethics and politics of global mental health and the internationally-driven expansion of psychiatric services in contexts like Sri Lanka, the growth of biomedically-oriented mental health services raises concerns about medicalization and about the possibility that such services may undermine other forms of expertise and cultural understanding, including established medical systems like Ayurveda or religious healing practices. On the one hand, the psychiatric community remains committed to evidence-based, clinical models of treatment, shaping state-based mental health care in this direction. On the other hand, the consolidation of the mental health and psychosocial support (MHPSS) field in the aftermath of the tsunami and in the post-war context has meant not only the systematic documentation and dissemination of local expertise drawn from those experiences but also the foregrounding of critical perspectives in the conceptualization and implementation of mental health and psychosocial services in the country as a whole (Galappatti, 2014).

The specific history of mental health services in Sri Lanka has shaped critical expert discourses in the MHPSS field. As key practitioners in Sri Lanka have noted, the tension between approaches emphasizing clinical or therapeutic interventions and those that focused on social and

¹ Government statistics from 2016, for example, indicated a wide disparity between poverty rates in urban (1.6%), rural (4.3%) and estate (8.8%) sectors. Similarly, there were large disparities among the provinces and districts. At the district level, the highest rates of poverty were in areas directly impacted by the civil war (Sri Lanka Department of Census and Statistics, 2016). The Central Province had an official poverty rate of 5.4%, which was also higher than the national rate, and a relatively high number of people living in poverty. Notably, even at the time these statistics did not accurately reflect levels of economic vulnerability in the population (CEPA, 2016)

² The debt crisis in Sri Lanka has led to a doubling of the national poverty rate from 13.1% in 2021 to nearly 26% in 2022 and 2023 (World Bank, 2024).²

³ It is important to emphasize that the problem of meaningful and effective mental healthcare is not unique to LMICs and reflects deeper problems with psychiatric systems of care across high and low and middle income settings (e.g., Luhrmann and Marrow, 2016; Mendenhall, 2016).

material support mirrored the theoretical debate that had been unfolding in international circles about “trauma” versus “resilience” approaches (Galappatti, 2003, 8; Samarasinghe, 2002). The critique of trauma-focused clinical approaches questioned not only the effectiveness and applicability of the trauma model and PTSD diagnosis in cross-cultural contexts but also its political implications. Critics pointed out the individualizing and depoliticizing effects of the early clinical approach that ignored the social determinants of distress and questioned the hegemonic status of Western medical knowledge, which often displaced local practices and resources that could aid recovery (Summerfield, 1999; Pupavac, 2001). In Sri Lanka, however, the debate had emerged not as a set of conceptual issues but as a response to the practical realities of psychosocial service provision in the context of armed conflict and with highly socially, culturally and religiously diverse communities (Galappatti, 2003; Samarasinghe, 2014). Counseling or “talk-therapies” did not always resonate with clients who tended to prioritize their social and material needs; there were limited resources for training and supervision of counselors; and the reality of ongoing conflict made therapeutic activities potentially risky (Galappatti, 2014).

These dilemmas—materialized against the backdrop of extreme loss and economic devastation among war-affected communities—led Sri Lankan practitioners and scholars in the MHPSS field to develop approaches that would meaningfully integrate psychological, social and material dimensions of people’s lives and experiences into service provision. This integration is the core principle of “psychosocial” interventions, which have been broadly incorporated into humanitarian and long-term development programs in the country (Abeyasekera and Wettasinghe, 2016, 2). Many of these drew on community-centered and counseling approaches, linking together psychological and social supports through the training of community volunteers and coordinated referral networks. Other psychosocial interventions included addressing practical needs such as housing; initiatives to improve existing services; family tracing and reunification; and facilitating traditional or religious healing (Abeyasekera and Wettasinghe, 2016; Galappatti, 2003).

Encompassing diverse activities and various models of integration in service provision, the mental health and psychosocial care framework is underpinned by values, which ideally inform these interventions and which respond to broader debates and critiques within the fields of humanitarian assistance and development (Galappatti, 2014; Abeyasekera and Wettasinghe, 2016, 33). Articulations of psychosocial approaches in the literature reflect a commitment to person-centered and justice-oriented practice grounded in respect for the communities and individuals receiving support. This may include consultative processes that ensure interventions are responsive to community needs as well as efforts to minimize social hierarchies that might otherwise characterize relief or development work (PADHI, 2008; Marecek et al., 2011). The Psychosocial Assessment of Development and Humanitarian Interventions (PADHI) framework and wellbeing assessment tool developed by a team of social scientists at University of Colombo exemplifies this orientation and offers a guide for development practitioners to put it into practice. Psychosocial approaches further recognize the social or collective nature of suffering and the importance of drawing on and strengthening family and community ties. This reflects the broader understanding that for many people across Sri Lanka’s communities, family life and social relationships are a constitutive element of wellbeing (PADHI, 2008; Somasundaram, 2014).

Related to the above, MHPSS interventions must be culturally responsive. In one sense, this means understanding how cultural sensibilities and values may shape individuals’ experiences of distress and priority needs. In another, it means a pluralistic approach that draws on local medical traditions as well as the social, cultural and spiritual resources that already exist within communities and that can be mobilized in healing and recovery (e.g., Ganesan, 2011; Somasundaram, 2014). Attention to social and cultural realities and expectations again suggests ground-up rather than top-down approaches in designing and

implementing interventions. Importantly, the difference is nuanced beyond broad ethno-religious group identities to encompass localized experiences of conflict or disaster, political considerations, and the social dynamics within specific communities.

The principles outlined above reflect the critical perspective embedded in the work of scholars and practitioners who have been at the forefront of coordinating, organizing, and documenting MHPSS interventions in Sri Lanka and beyond (Samarasinghe, 2014; Somasundaram, 2014; Galappatti, 2014; Appuhamilage, 2018; Ganesan, 2011; Amarasuriya and Abeyasekera, 2010). Particularly in the post-tsunami and post-war contexts, the emphasis on community participation and critique of clinically-oriented interventions was contiguous with a concern for addressing the political, economic and social dimensions of distress at the community level and in relation to the wider national context (Somasundaram, 2014; Samarasinghe, 2014; Herath et al., 2012). This recognition of structural determinants of distress has been increasingly foregrounded in studies that seek to capture in clinical terms the relationships between particular social conditions or experiences and symptoms of conditions like PTSD or trauma-related depression or anxiety (Jayawickreme et al., 2017; Jayasuriya et al., 2016; Thomas et al., 2022).

Whereas conflict- and disaster-related trauma as well as post-war recovery make urgently visible the realities (social, economic and political) that engender and exacerbate psychological distress, this point is equally salient in relation to “everyday” forms of distress as indexed by the notion of common mental health disorders, which have been of particular focus in the broader GMH agenda and the concern with access to mental health services (Weaver et al., 2023). Efforts to close the “treatment gap” through expansion of community-based mental healthcare have rested on the pillar of evidence-based medicine and the largely biomedical paradigm this reproduces in practice. The salient gap, however, is the disjuncture between medicalized notions of mental health and the complex realities of psychosocial distress across diverse local contexts. Beyond the lack of (biomedical) mental health services in LMICs highlighted by GMH advocates, critics point to the social determinants of mental health (Jain and Jadhav, 2009; Mills, 2014; Shukla et al., 2012; Srinivasan et al., 2023), to the routinized forms of structural violence that communities and individuals face (Kottai, 2020; Das, 2015), and to the deleterious effects of the medicalization of distress (Kottai and Ranganathan, 2020).

As Weaver et al. (2023) illustrate in the context of India, medicalized or psychologized understandings of distress may not capture how ordinary people theorize their experiences, typically linking them to precisely those social and economic realities which mental healthcare tend to elide. This insight resonates strongly with the critical perspectives of mental health practitioners, patients and clients in Sri Lanka. As the authors point out, “While counseling interventions often emphasize interrogating one’s inner feeling states and focusing on changing thought patterns, women in the present study were much more interested in changing disadvantageous circumstances that led to their distress in the first place” (Weaver et al., 2023, 6). Even if participants who had symptoms of common mental health disorders accessed such care, it “might not resonate with their own models for the root causes of their distress” (6). As I show below, psychological counselors in Sri Lanka’s state sector recognized and sought to address the disconnect between traditional models of counseling and their clients’ complex psychosocial needs. Similarly, hospital-based mental health providers critically described shortcomings of the medicalized model of care in which they were forced to operate and the need for more resources to meaningfully address patients’ distress. Their perspectives point to needed institutional and policy changes that would prioritize not only access to medical treatment but also to various forms of psychosocial care that are responsive to ground realities of social and economic vulnerability.

3. Methods

The ethnographic study of government mental health services was carried out between April 2018 and January 2020 in Kandy District, Central Province. The fieldwork engaged multiple groups of participants, sites and data collection methods as part of a broader ethnographic project. The below discussion is based on 30 semi-structured, audio-recorded interviews with 15 government counseling officers. These were complemented by ongoing informal conversations with counsellors, social service officers and former clients in the course of participant observation. This included 33 visits at three DS offices and participation in 43 community mental health awareness programs across the district.

Between June 2019 and January 2020, I conducted 14 observations at an outpatient psychiatric clinic. This included a brief interview survey with 42 randomly selected patients and carers. Individuals who appeared not to be able to give informed consent were excluded. To minimize potential for discomfort, the survey did not inquire about respondents' diagnoses or problems, though many people did share. Questions focused on details such as length of attendance at the clinic, medication use, perceptions of improvement, and access to other forms of treatment (e.g. counseling, spiritual). Not all respondents answered all questions asked; many shared information beyond what was asked. The conversations lasted from 5 to 20 min. The clinic observations were followed by interviews with two psychiatric social workers, six community psychiatrists, and a psychiatric nurse.

Nearly all interactions took place in Sinhala with a limited number of interviews in English. The research was carried out with the assistance of a Sinhala-speaking research assistant, Ann Rebecca Roche, who subsequently assisted with transcription and interpretation of data. The transcribed interviews, lectures and mental health awareness programs were analyzed in Sinhala with selected quotes translated into English for the purpose of writing up the analysis. Translations were carefully considered with Ann Rebecca Roche and a second qualified Sinhala speaker to capture as much as possible nuances of meaning.

Drawing on modified grounded analysis (Charmaz and Mitchell, 2001), data analysis involved close, iterative reading and manual coding of interview transcripts and field notes, guided by conceptual frameworks in critical medical anthropology (e.g., social determinants of distress, class, and medicalization) as well as categories emerging from the data (e.g., coordination of services, chronicity, institutional hierarchies). The short survey interviews with patients and carers were analyzed for content with answers collated and listed in tables (e.g., how many clients reported accessing counseling). Looked at holistically, responses were then analyzed for core themes in patients' and carers' descriptions of the illness experience, treatment, and life circumstances. Ethics approval was obtained from the Ethics Review Committee for Social Sciences and Humanities at the University of Colombo (Reference no. ERCSSH/18/46).

4. Public mental health services and the challenge of addressing psychosocial needs

4.1. State-based psychological counseling services

The Sri Lankan government began to incorporate psychological services into public education, the military and social welfare programs from 2005 onwards.⁴ As part of the latter, a nation-wide, public counseling service was implemented through the existing structure of

district- and divisional-level government services. By 2017, there were 435 Counseling Officers dispersed across the country's 341 local Divisional Secretariats, though services have not been evenly distributed—rural divisions distant from urban centers as well as those in the northern and eastern regions of the country are often underserved. The counselors provide free individual and family counseling to local residents facing myriad psychosocial and psychiatric problems, but they also do outreach in their respective communities, ranging from urban centers to remote villages. Eighteen of the 20 government counselors in Kandy District identified as Sinhala Buddhist and two as Muslim. The counselors' training involves a four-year degree (often in psychology or sociology) combined with a counseling diploma course. Counselors also participate in ongoing professional development, the most robust of which has been through a partnership between the government and The Asia Foundation (TAF), an international organization that has played a critical role in building up both non-governmental and state mental health services island-wide (The Asia Foundation, 2016; The Asia Foundation, 2018).

As the only mental health professionals in the social service setting of the DS office, the counselors work with clients facing distress diagnosed as depression, anxiety, bipolar disorder or obsessive-compulsive disorder (OCD) as well as issues such as addiction, family conflicts, domestic violence, and children's behavioral problems. According to a country-wide survey of 314 government counselors, the most common problems reported included alcohol and drug dependence, marital and parent-child issues, child behavioral problems, mild psychological or mental health disorders and economic problems, followed by a variety of other less common issues (The Asia Foundation, 2018, 44).

In interviews, counselors pointed to family and economic problems as well as mental health disorders as common presenting issues. In their analysis, such problems were often framed as a tangle of intersecting and mutually reinforcing stressors that highlighted clients' social and economic vulnerabilities. These were in turn understood to impact emotional and physical wellbeing. While some cases were understood in narrower terms as a specific psychological or psychiatric condition (e.g., social anxiety), more often clients faced multifaceted problems that linked psychological distress to socio-economic and interpersonal troubles. Moreover, although clients experiencing more severe symptoms (e.g., suicidal ideation, prolonged sleeplessness) were likely to be referred for psychiatric care, the counselors sought to concurrently address the social and economic dimensions of their situation where relevant. While the counselors emphasized their psychological expertise and attention to clients' emotional distress as something that distinguished them from other social service providers, working in the government sector also meant that they were part of and could tap into a broader network of services to support clients in holistic ways. This is how one counselor characterized her work:

When we take on the role of a counselor, it means that we need to help people with these problems. We need to find places that can give support. Now in our role ... they [clients] come here, talk with us and leave. We help make them feel relaxed, we give them empathy and with that they might feel relaxed. However, there are some other things we must do to go beyond that. In the office there are many other officers. They help with self-employment, they give different benefits, there are various things like that. [...] We cannot help (ගොඩ ගන්න, *goDa ganne*) a person alone, only through counseling. There are some other things that need to come with counseling. If those needs are fulfilled then only the person becomes positive. Sometimes even just one session is enough. The reason is that they have received the support they needed. [Interview, February 2019]

Another counselor echoed this idea when discussing how counseling differs from other social services. Reflecting a common theme, she emphasized that the counselors are not able to directly give clients benefits. However, she then went on to describe how they connect clients with appropriate support, suggesting that such support is integral to

⁴ It is significant that counseling services were established in the aftermath of the 2004 Tsunami. Before this, in the midst of a protracted civil war that continued until 2009, the state refused to acknowledge the psychological impacts of war, restricting and surveilling the work of psychosocial organizations and blocking the incorporation of mental health into state services.

the counseling service itself:

For example, if there is a home program, we tell [the relevant officers], one of our clients, we have talked to them a lot, they need this help, please take a look. He's this kind of person [i.e., qualifies for the assistance]. Then it's convenient to get approval since we have the client's details. He's an appropriate candidate. [...] So we can help from that side. We don't have anything material to give. [...] But we can connect clients with the relevant officers. Counseling cannot happen on its own. You need everyone's support. You cannot help (ගොඩ ගන්න, *goDa ganne*) a person alone. You definitely [also] need the family's support." [Interview, July 2019]

Moreover, clients' problems were understood to be layered such that the immediate concern a client presents to the counselor may be connected with many other issues, which are not made explicit right away. In such situations providing practical or material support could create an opening for further counseling support. The same counselor quoted above, offered the following example:

The client might come to get some help for her child. She has a child who wants to study but cannot, let's say because of economic problems. She comes for that but talking to her that is not the only thing; there is so much more. There are so many different problems affecting her. From here to "bring her along" [to continue the counseling process], sometimes if we can, if it's possible- we can't always give clients everything, there are not always funds. But if we can, we help the child [e.g., financially or with a scholarship]. After that we can develop a relationship with the family, with that trust gained. [Interview, December 2018]

The counselors' interventions thus expanded beyond the counseling session, mobilizing existing services such as welfare benefits, loans, and employment support to address clients' and their families' material insecurity. As the interviews quoted above and direct observations of interventions suggest, this meant actively collaborating with other officers in the social care units of the DS office; mediating on behalf of clients; and regularly following up on these cases.⁵ At times, the counselors and other officers drew on their own resources by collecting donations or drawing on personal networks to help a client secure school or medical supplies, support for their small business, or even employment. Though the counselors theorized clients' economic and psychosocial problems in diverse ways, their discourse and practice reflected an understanding of psychological distress that folded material, relational, and practical dimensions of everyday life *into* it rather than seeing these as contributing factors.⁶

Echoing studies on resilience and cultural understandings of distress in Sri Lanka, the counselors framed wellbeing more broadly in terms of one's relationships, capacity to fulfill social and familial obligations, and sense of material security (Fernando, 2008, 2012a, 2012b; Jayawickreme et al., 2009; PADHI, 2008). Material support become especially important when working with clients facing psychosocial disability or other constraints on their capacity to earn as well as women clients without independent income who faced economic vulnerability as a result of ruptures in kin-based support. In line with state policies and services, this often meant encouraging clients to engage in some form of

home-based or self-employment alongside securing benefits whenever possible.

Insofar as the counselors' efforts remained constrained by the state social service structure and did not always challenge social hierarchies, their practice did not necessarily reflect more critical and transformative approaches that may characterize psychosocial disability activism and grassroots mental health work (e.g., Marecek et al., 2011; Srinivasan et al., 2023). Nevertheless, their interventions were driven by explicit concern with the social and material dimensions of their clients' distress as well as deep sensitivity to culturally-inflected understandings of care. In this sense, they went against the grain of dominant psychotherapeutic approaches, which have been critiqued for failing to attend to the role of class and economic precarity in clients' experiences (Berzins et al., 2018; Goodman et al., 2010; Thompson et al., 2012).

At the same time, case descriptions collected in the course of interviews attested to the limits of the care the counselors were able to provide. The economic problems that many clients faced meant that some simply could not continue to return for sessions. The counselors adjusted to this by extending first sessions and by trying to help clients as much as possible in that first encounter. They also conducted home visits and maintained contact with clients over the phone. Some of the most difficult cases involved clients experiencing domestic violence as well as those who were caring for children with disabilities. This reflects not only economic inequalities but also the state's failure to ensure more expansive infrastructures of care that go beyond individually-focused poverty alleviation programs or minimal cash and disability benefits. There are not enough shelters and services for women experiencing domestic violence (Kodikara and Piyadasa, 2011) and clients as well as carers of adults and children with psychosocial disabilities face limited specialized care and educational facilities (Caldera et al., 2023; Higashida, 2014, Muttiah et al., 2016). Similarly, in the face of an aging population, families are mostly left to their own devices to ensure care while negotiating the imperatives of paid work (Gamburd, 2021, Muditha et al., 2018). It was especially in such situations that counselors felt helpless to provide effective support.

4.2. Outpatient and community-based psychiatric services

This pattern of inadequate infrastructures of care was foregrounded at the intersection of counseling and psychiatric services. Even as Kandy is a relatively well-resourced district, and especially the urban areas where this research was focused, observations and conversations with doctors and patients at an outpatient psychiatric clinic in a divisional hospital close to Kandy City offered a complicated picture.

Patients' stories suggested that treatment prioritized psychotropic medication even when distress appeared directly linked to difficult life circumstances that remained unresolved (e.g., an abusive relationship or a difficult economic situation).⁷ The situation is not as simple as social suffering being routinely medicalized. In patients' narratives, distress was not always or necessarily clearly linked to socio-economic or family problems, and when this was the case, the doctor often recognized patients' broader struggles, referring some for counseling or social services in addition to medication. Nevertheless, of the 30 people who responded to the question, sixteen had either never heard of counseling or heard of it but never received a referral. Of the fourteen respondents who did, nine went for only one session. The evident chronicity of distress and medication use and the sense that for many respondents meaningful recovery remained out of reach brought into relief the limitations of a predominantly medical mode of treatment and of the available supportive services. While doctors embraced medical treatment, this

⁵ Except in two cases, I did not observe individual counseling sessions as part of the research. However, I participated in home visits, witnessed case conferences and observed informal interactions between counselors and clients at the DS offices.

⁶ This dimension of the counsellors' work has raised questions among some mental health experts about the counsellors' role and the scope of their work as well as about the implications their approach has for the therapeutic goal of enhancing client agency. While this is beyond the scope of this discussion, I explore these issues in depth in Chapter 4 of my dissertation (2023) as well as in a forthcoming publication based on the same (Augustyniak, 2025).

⁷ Data on prescribed medications was not collected. However, whenever respondents named medications these were psychotropics, and the patients' self-reported side effects were consistent with the use of antipsychotics, anxiolytics, and antidepressants.

seemed not merely a professional commitment but also a product of resource constraints—in 2019, on the observed clinic days, the psychiatrist typically saw more than 70 patients in one morning.

The practitioners who shared their observations with me offered critical perspectives on the contradictions and gaps in the available system of community-based mental health care.⁸ For example, a mental health practitioner affiliated with one of the district hospitals, commented on the prevalence of depression and medication use in the context of some patients' difficult social and economic circumstances. Noting that alternatives like counseling, exposure therapy or CBT are less common in the hospital setting, he attributed the over-reliance on medication to hospitals not having enough personnel with the relevant therapeutic expertise. Questioning whether this is the right thing to do, he pointed out that while patients are receiving treatment, "the economical problems, [and] other things are not being solved ... so everybody [has] depression. So many people have a lot of problems actually. Because we don't know how-where do we go-how do we solve the problem?" Speaking about a patient whose acute distress was obviously linked to serious economic hardship, another practitioner noted that as providers, even they don't see clear solutions and medication becomes a means to reduce symptoms in the hopes that, with supportive services like counseling, the patient may feel more capable to face her problem.

While technically the thick network of referrals should ensure that patients are able to access counseling, welfare or employment support and assistance with housing, such support often appeared inadequate to the need. As another practitioner quipped, "everything is there [...] but no solutions." In his view, as well as in the eyes of other practitioners in the district, the challenges are multiple. Coordination of services remained a problem with tasks in a given case at times passed from officer to officer "like a ball." At times, the allocation of benefits or the transfer of a patient from the hospital to a long-term facility could take weeks. Specialized and especially residential care facilities were limited, leaving much of the responsibility of care to kin. In one observed case, a person with serious psychosocial disability was living alone in deleterious conditions, their family providing only minimal care. A team consisting of a medical officer of mental health, a counselor, and a psychiatric social worker conducted multiple home visits and intervened to try to secure not only medical treatment but also safer housing conditions and a more supportive family and community environment, enabling ongoing care. Because of the family's lack of cooperation and previous neglect as well as concerns about the client living alone, the hope was that the client might be able to stay in a long-term care facility. Unfortunately, finding such a facility proved impossible. A follow up conversation with the involved counselor revealed that ultimately the client returned to living alone, reliant on the minimal and reluctant care of their relations. This single case does not offer any insight into the frequency of such outcomes. Nevertheless, it starkly illustrates the extent to which the system depends on kin to take responsibility for long-term care while providing very limited options when this care is not possible.

Just as for the counselors, inadequate facilities limited doctors' and social workers' efforts to support individuals with psychosocial disabilities and their families. Competing demands amidst limited resources meant that social support did not always materialize. As the practitioner quoted above put it, "In our capacity we are connected to all these other services—education department, DS office, social service—but the problem is that sometimes they give the support and sometimes they don't." Moreover, professional hierarchies in hospital settings mean that counsellors and social workers find themselves constrained in making decisions that would facilitate coordination of services. Finally,

most of the social services are provided through the DS office, yet as noted above, for very low-income clients, access can be limited by cost of travel and loss of daily wages.

4.3. Systemic barriers to state-based care

The barriers that limited resources create are exacerbated by the ways in which social inequalities are woven into public social service provision more broadly. In the Sri Lankan context this includes the state-based social and economic support that mental health providers seek to offer their clients and patients through referrals. Scholars examining state-society relations in Sri Lanka have documented the various forms of exclusion and politicization that shape citizens' access to healthcare, education, infrastructure and social welfare (Abayasekara and Arunatilake, 2018; Jegathesan, 2018; Godamunne, 2016; Gunasekara et al., 2019; Jeyasankar and Ganhewa, 2018; Kodikara, 2018; Kumar et al., 2022; Liyanagunawardena, 2023; Ramesh and Nanthakumar 2018; Romeshun and Vagisha, 2014; Sarma et al., 2018; Uyangoda, 2012; Emmanuel and Saroor, 2022). In a context marked by high levels of socio-economic inequality as well as a history of conflict and ethno-nationalist politics, communities marked by lower class, caste, often though not necessarily minority ethno-religious identity, lower status labor, disability or other marginalized identities may experience state care very differently than those with relatively more privileged status.⁹

Such issues emerged in one division where the counselor had collaborated with the Samurdhi Officer (SO) and Development Officers (DO) to "uplift" a local, predominantly Tamil Hindu, low income community that had been stigmatized with stereotypes of criminality and drug use. The officers had apparently tried to set up a mobile service so that residents can have easier access to administrative procedures like applying for a national ID card. As recounted by the counselor, the project was also meant to build a better relationship with the community as the residents were reluctant to access social support via the DS office. Yet the mobile service was cancelled three times, affirming the community's perception that the government does not help. One of the officers involved acknowledged that caste discrimination played a role in residents' apparent lack of trust in state services, yet in reflecting on the failure to serve the community, they emphasized the residents' purported attitude of resignation as the problem, effectively erasing the reality of exclusion.

During a follow up visit, one of the families who had participated in the government officers' programs offered a different analysis. Although they appreciated the DO's and other officers' efforts, they were skeptical of state support. This was in part because the community apparently had a very fraught relationship with the village officer who, as the family we visited alleged, actively blocked other officers' efforts to implement development projects in the area. The implication was that this was on grounds of ethnic prejudice. Without revealing any details, the counselor later also intimated that there had been personnel issues in implementing projects in this community.

Language presents another example. In Kandy District, Tamil speakers face local administrations run primarily in Sinhala with relatively few civil servants proficient in both official languages. In the counseling service, this issue appears pronounced. In 2013, for example, of all surveyed counselors in the government sector, 85% were very proficient in Sinhala while fewer than 21% were proficient in Tamil (The Asia Foundation, 2013). At the time of my research, in Kandy District only two counselors were able to conduct sessions in Tamil despite the fact that Tamil-speaking minorities comprise approximately 25% of the district's population. Despite the state's efforts to address this disparity,

⁸ Due to the small number of providers interviewed within the professional network in Kandy District, to protect interlocutors' anonymity I do not specify job titles, referring to all respondents as "practitioners."

⁹ Uyangoda (2012), for example, offers a complex portrait of the politics of access to state services among caste-marginalized Sinhala-speaking communities in Kurunegala.

the observed situation suggests that much more needs to be done.

Across national contexts, the forms of exclusion that shape citizens' access to state services trace the experience of structural violence and dispossession at the intersections of class, gender, ethnicity, religion, disability, caste and other highly localized identities and relationships. Their impacts are contiguous with historical forms of state violence and dispossession that marginalized and minority communities have experienced and continue to face in various ways. This broad point highlights the political dimension of mental health and psychosocial services, which manifests itself not only in the ways that political and economic realities structure experiences of distress but also in the ways they constrain the possibilities for meaningful support.

5. Discussion

The assertion that the expansion of mental health services should go hand in hand with the expansion of social protections is a logical outcome of the widely shared understanding that mental health is entwined with structural realities shaping access to basic needs like safety, income, housing, education, childcare and healthcare. Yet, even as global mental health advocates recognize the significance of social and economic determinants of distress, on the ground realities in some contexts suggest that biomedical interventions tend to be prioritized while social and economic support remains a challenge. Although Sri Lanka has seen significant expansion of access to mental health care over the last 20 years, these developments have focused primarily on psychiatric and, to a lesser extent, psychological services. While access to medical mental health care remains uneven and has been negatively impacted by the 2022 economic crisis, the challenges in the public mental health sector point to the medicalized nature of available care and its limits and contradictions.¹⁰

In theory, medical services should be coordinated with the broader social services system, thereby ensuring that clients can also access needed social, economic and psychological support. However, ethnographic data from Kandy district suggests that in practice—even in a relatively well-resourced urban context—such coordination is strained and limited social protections and infrastructures of care make it difficult to address the economic and social vulnerabilities that often exacerbate, or even stand at the root of, clients' and patients' distress. For the government counsellors, the recognition of many clients' difficult life circumstances and material needs articulated with broader cultural understandings of helping relationships and care, leading them to fold economic and social support into their interventions in at times quite direct ways. In medical settings, providers such as community-based psychiatrists (MOMHs), psychiatric nurses and social workers—were similarly acutely aware of patients' complex psychosocial needs even as medical interventions remained the predominant response. Across these overlapping contexts, the constraints of an under-resourced and highly individualized system of social protections and structural barriers to accessing social services shaped the limits of mental health care. In this sense, in the context of my observations, medicalization did not necessarily mean that psychosocial distress was viewed through the lens of medical categories but rather that medical interventions were often the one form of care that could be offered.

The perspectives of Sri Lankan mental health practitioners highlighted above do point to systemic changes that could improve their ability to provide psychosocially-informed care. Examples include better coordination of services at the district level, a less hierarchical professional structure, and above all, more specialized care facilities. At the same time, more in-depth, qualitative research is needed to understand the diverse perspectives, experiences, and expectations of clients and patients and their families. This includes keen attention to not only

socio-economic but also cultural dimensions of experience and openness to diverse visions of what constitutes meaningful or ethical care from both client and provider perspectives (Tribe, 2014).

6. Conclusion

The 2022 debt crisis in Sri Lanka and its ongoing effects has been laid over existing vulnerabilities and struggles of communities across the country, including those that continue to face long-term impacts of war-time violence. Mental health discourses on both global and local scales have tended to depoliticize distress by focusing on symptoms and on the expansion of access to largely biomedical treatments. Especially in times of crisis such as the Covid-19 pandemic or economic collapse—situations of acute but “ordinary” suffering—there is often a slippage between articulations of the need for more mental health services (i.e., psychiatric and psychological interventions) and the notion that such services could be the primary response to widespread psychosocial distress in the absence of social and economic forms of support. The connection between material, social and psychological wellbeing that so many practitioners and ordinary people understand as part of everyday common sense becomes obscured, further entrenching the medicalization of distress. This connection should be explicitly emphasized in government policy as well as in the everyday public discourse on access to mental health services, foregrounding the political, social and economic dimensions of wellbeing at multiple scales—from state- and community-based psychosocial support (including medical and psychological) to the broader public policies, institutions and systems that shape possibilities for psychosocial wellbeing. Such emphasis could be especially resonant in the current moment of political change and possibility in Sri Lanka, even as the country continues to grapple with the economic and social impacts of the 2022 crisis.

Declaration of competing interest

The authors declare that they have no known competing financial interests or personal relationships that could have appeared to influence the work reported in this paper.

References

- Abayasekera, A., Arunatilake, N., 2018. School-level resource allocation and education outcomes in Sri Lanka. *Int. J. Educ. Dev.* 61, 127–141.
- Abeyasekera, Asha L., Wettasinghe, K., 2016. Mental health and psychosocial support (MHPSS) services in Sri Lanka: a review of the literature 2005 – 2015. In: *Transitions In Mental Health And Psychosocial Services In Sri Lanka 2004-2015: A Decade In Review*, Colombo: the Asia Foundation.
- Amarasuriya, H., Abeyasekera, A., 2010. Why aren't we empowered yet?. In: *Charting Pathways to Gender Equality: Reflections and Challenges*. Center for Women's Research (CENWOR), Colombo, pp. 107–138.
- Appuhamilage, Udeni, 2018. Beyond a psychologistic trauma: a sense-based approach to the study of affect. *Qual. Res. Psychol.* <https://doi.org/10.1080/14780887.2018.1545062>.
- Augustyniak, Nadia. 2025. When the counsellors give: material support and therapeutic agency in state-based psychological counselling services in Sri Lanka. In *Towards An Anthropology Of Psychology*, Mikkel Kenni Bruun and Rebecca Hutten, eds. London: Berghahn Books, forthcoming.
- Berzins, S., Babins-Wagner, R., Hyland, K., 2018. Relationship of employment status and socio-economic factors with distress levels and counseling outcomes during a recession. *Counsel. Psychother. Res.* 18 (2), 122–132.
- Caldera, A.V., Wickremasinghe, R., Munasinghe, T.U., 2023. Availability of rehabilitation services for communication disorders in Sri Lanka: a Cross-sectional Survey. *BMJ Open* 13, e071620. <https://doi.org/10.1136/bmjopen-2023-071620>.
- CEPA, 2016. Sri Lanka's poverty line: more than just a number. Center for poverty analysis (CEPA). Electron. Doc. <http://www.cepa.lk/blog>. (Accessed 7 June 2016).
- Charmaz, K., Mitchell, R.G., 2001. Chapter 11: grounded theory in ethnography. In: Atkinson, Paul, Coffey, Amanda, Delamont, Sara, Lofland, John, Lofland, Lyn (Eds.), *Handbook of Ethnography*. Sage, Thousand Oaks, CA.
- Das, Veena, 2015. *Affliction: Health, Disease and Poverty*. Fordham University Press, New York.
- Emmanuel, Sarala, Saroor, Shreen, 2022. Experiences of gendered norms and mobilizing for rights of women living with disabilities in the post-war context in Sri Lanka. *Front. Soc.* 7, 715240. <https://doi.org/10.3389/fsoc.2022.715240>.

¹⁰ Jayawardena et al. (2023) discuss the deleterious impacts of the economic crisis on medication adherence more broadly.

- Fernando, Gaithri, 2008. Assessing mental health and psychosocial status in communities exposed to traumatic events: Sri Lanka as an example. *Am. J. Orthopsychiatry* 78 (2), 229–239.
- Fernando, Gaithri, 2012a. Bloodied but unbowed: resilience examined in a south asian community. *Am. J. Orthopsychiatry* 82 (3), 367–375.
- Fernando, Gaithri, 2012b. The roads less traveled: mapping some pathways on the global mental health research roadmap. *Transcult. Psychiatr.* 49 (3–4), 396–417.
- Fernando, Neil, Thirupathy, Suveendran, Chithramalee, de Silva, 2017. Decentralizing provision of mental health care in Sri Lanka. *WHO South-East Asia J. Public Health* 6 (1), 18–21.
- Galappatti, Ananda, 2003. What is a psychosocial intervention? Mapping the field in Sri Lanka. *Intervention* 1 (2), 3–17.
- Galappatti, Ananda, 2014. Trauma and beyond: the evolving field of mental health and psychosocial work in Sri Lanka. In: Somasundaram, Daya (Ed.), *Scarred Communities: Psychosocial Impact of Man-Made and Natural Disasters on Sri Lankan Society*. Sage Publications, New Delhi.
- Gamburd, Michele Ruth, 2021. *Linked Lives: Elder Care, Migration and Kinship in Sri Lanka*. Rutgers University Press, New Brunswick, NJ.
- Ganesan, Mahesan, 2011. Building up mental health services from scratch: experiences from East Sri Lanka. *Intervention* 9 (3), 359–363.
- Godamunne, Nayana, 2016. Can social protection build state legitimacy? Perspectives from post-war Sri Lanka. *Int. Relat. Diplom.* 4 (11), 703–717.
- Goodman, L.A., Smyth, K.F., Banyard, V., 2010. Beyond the 50-minute hour: increasing control, choice, and connections in the lives of low-income women. *Am. J. Orthopsychiatry* 80 (1), 3.
- Gunasekara, Vagisha, Surenthiraraj, Rebecca, Tilakaratne, Praveen, 2019. Services and legitimacy: exploring the everyday experiences of the state in Sri Lanka. *Researching Livelihoods and Services Affected by Conflict*, Working Paper 79. Secure Livelihoods Research Consortium in Collaboration with Social Scientists' Association. Colombo.
- Herath, Dhammika, Silva, Kalinga Tudor, 2012. In: *Healing the Wounds: Rebuilding Sri Lanka after the War*. International Center for Ethnic Studies, Colombo.
- Higashida, Masateru, 2014. Local resources of disabled people in Sri Lanka: action research on community-based rehabilitation programme. *Soc. Anthropol.* 2 (4), 159–167. <https://doi.org/10.13189/sa.2014.020404>.
- Jain, S., Jadhav, S., 2009. Pills that swallow policy: clinical ethnography of a community mental health program in northern India. *Transcult. Psychiatr.* 46 (1), 60–85.
- Jayasuriya, D., Jayasuriya, R., Tay, A.K., Silove, D., 2016. Associations of mental distress with residency in conflict zones, ethnic minority status, and potentially modifiable social factors following conflict in Sri Lanka: a nationwide cross-sectional study. *Lancet Psychiatr.* 3 (2), 145–153. [https://doi.org/10.1016/S2215-0366\(15\)00437-X](https://doi.org/10.1016/S2215-0366(15)00437-X).
- Jayawardena, R., Kodithuwakku, W., Sooriyaarachchi, P., 2023. The impact of the Sri Lankan economic crisis on medication adherence: an online cross-sectional survey. *Dialog. Health* 2, 100137.
- Jayawickreme, N., Jayawickreme, E., Goonasekera, M.A., Foa, E.B., 2009. Distress, wellbeing and war: qualitative analyses of civilian interviews from north eastern Sri Lanka. *Intervention* 7 (3), 204–222. <https://doi.org/10.1097/wtf.0b013e328334636f>.
- Jayawickreme, N., Mootoo, C., Fountain, C., Rasmussen, A., Jayawickreme, E., Bertuccio, R.F., 2017. Post-conflict struggles as networks of problems: a network analysis of trauma, daily stressors and psychological distress among Sri Lankan war survivors. *Soc. Sci. Med.* 190, 119–132. <https://doi.org/10.1016/j.socscimed.2017.08.027>.
- Jegathesan, Mythri, 2018. *Tea and Solidarity: Tamil Women and Work in Postwar Sri Lanka*. University of Washington Press, Seattle, WA.
- Jenkins, R., Mendis, J., Cooray, S., Cooray, M., 2012. Integration of mental health into primary care in Sri Lanka. *Ment. Health Fam. Med.* 9 (1).
- Jeyasankar, Vasuki, Ganhewa, Savini, 2018. Making Ends Meet: Women's Livelihoods in Post-War Sri Lanka. International Center for Ethnic Studies (ICES), Colombo. <http://ices.lk/wp-content/uploads/2018/03/Making-Ends-Meet-Women%E2%80%99s-Livelihoods-in-Post-War-Sri-Lanka.pdf>.
- Kodikara, Chulani, 2018. Doing This and That: Self-Employment and Economic Survival of Women Heads of Households in Mullaitivu. International Centre for Ethnic Studies (ICES), Colombo. <http://ices.lk/wp-content/uploads/2018/03/Doing-This-and-That-Self-employment-and-Economic-Survival-of-Women-Heads-of-Households-in-Mullaitivu.pdf>.
- Kodikara, Chulani, Piyadasa, Thiagi, 2011. Domestic Violence Intervention Services in Sri Lanka: an Exploratory Mapping 2009–2011. International Center for Ethnic Studies (ICES), Colombo.
- Kohrt, Brandon A., Mendenhall, Emily (Eds.), 2016. *Global Mental Health: Anthropological Perspectives*. Routledge, New York.
- Kottai, Sudarshan, 2020. Mental health and migrant workers. *Econ. Polit. Wkly.* 55 (31), 39–45.
- Kottai, Sudarshan R., Ranganathan, Shubha, 2020. Task-shifting in community mental health in Kerala: tensions and ruptures. *Med. Anthropol.* 39 (6), 538–552. <https://doi.org/10.1080/01459740.2020.1722122>.
- Kumar, R., Birn, A.E., Bhuyan, R., Wong, J.P.H., 2022. Universal health coverage and public-private arrangements within Sri Lanka's mixed health system: perspectives from women seeking healthcare. *Soc. Sci. Med.* 296, 114777.
- Liyanagunawardena, S., 2023. Wrangling for health: moving beyond 'tinkering' to struggling against the odds. *Soc. Sci. Med.* 320, 115725.
- Luhrmann, T.M., Marrow, Jocelyn, 2016. Introduction. In: Luhrmann, T.M., Marrow, Jocelyn (Eds.), *Our Most Troubling Madness: Case Studies in Schizophrenia across Cultures*. University of California Press, Berkeley.
- Marecek, Jeanne, Appuhamilage, Udeni, M.H., 2011. Present but unnamed: feminism and psychologies in Sri Lanka. In: Rugherford, A., Capdevila, R., Undurti, V., Palmay, I. (Eds.), *Handbook of International Feminisms*. International and Cultural Psychology, pp. 315–333.
- Mendenhall, Emily, 2016. The "Cost" of health care: poverty, depression, and diabetes among Mexican immigrants in the United States. In: Kohrt, Brandon A., Mendenhall, Emily (Eds.), *Global Mental Health: Anthropological Perspectives*. Routledge, New York.
- Mills, China, 2014. *Decolonizing Global Mental Health: the Psychiatrization of the Majority World*. Routledge, New York.
- Minas, H., Mendis, J., Hall, T., 2017. Mental health system development in Sri Lanka. In: Minas, H., Lewis, M. (Eds.), *Mental Health in Asia and the Pacific*. Springer, New York.
- Muditha, Muthucumarana, Wagachchige, Kerstin Samarasinghe, Elgán, Carina, 2018. Caring for stroke survivors: experiences of family caregivers in Sri Lanka – a qualitative study. *Top. Stroke Rehabil.* 25 (6), 397–402. <https://doi.org/10.1080/10749357.2018.1481353>.
- Muttiah, N., Drager, K.D., O'Connor, L., 2016. Special education in Sri Lanka: a snapshot of three provinces. *Disabil. Stud. Q.* 36 (2).
- PADHI, 2008. *A Tool, a Guide, and a Framework: Introduction to a Psychosocial Approach to Development*. Social Policy Analysis and Research Centre, University of Colombo, Colombo.
- Pupavac, Vanessa, 2001. Therapeutic governance: psycho-social intervention and trauma risk management. *Disasters* 24 (4), 358–372.
- Ramesh, R., Nanthakumar, A.R., 2018. "Including the excluded: plantation communities and local government services." special issue: local government, local governance, local democracy. *Law Soc. Trust Rev.* 29 (345), 42–53.
- Romeshun, K., Vagisha, Gunasekara Mohamed Munas, 2014. Life and debt: assessing indebtedness and socio-economic conditions of conflict affected housing beneficiaries in jaffna, kilinochchi and mullaitivu districts. Study Series No 7. Centre for Poverty Analysis (CEPA), Colombo.
- Samarasinghe, Gameela, 2002. Counseling vs. community-based approaches. Paper Presented at the National Conference on Mental Health. Sahanaya, Colombo, Sri Lanka unpublished.
- Samarasinghe, Gameela, 2014. Psychosocial interventions in Sri Lanka: challenges in a post-war environment. In: Somasundaram, Daya (Ed.), *Scarred Communities: Psychosocial Impact of Man-Made and Natural Disasters on Sri Lankan Society*. Sage Publications, New Delhi, pp. 346–396.
- Sarma, V., Licht, S., Kalugalagedera, T., 2018. Educational Inequalities in Sri Lanka: National Data and Local Perspectives on Access, Quality and Learning Outcomes. Center for Poverty Analysis. Colombo, Sri Lanka.
- Shukla, A., et al., 2012. Critical perspectives on the NIMH initiative 'grand challenges to global mental health. *Indian J. Med. Ethics* 9 (4), 292–293.
- Somasundaram, Daya, 2014. *Scarred Communities: Psychosocial Impact of Man-Made and Natural Disasters on Sri Lankan Society*. Sage, Los Angeles, London, New Delhi.
- Sri Lanka Department of Census and Statistics, 2016. Household income and expenditure survey poverty indicators. <http://www.statistics.gov.lk/Poverty/StaticInformation/PovertyIndicators-2016>. (Accessed 18 March 2021).
- Srinivasan, V., Jain, S., Kwon, W., Bayetti, C., Cherian, A.V., Mathias, K., 2023. What innovative practices and processes are used to deliver psychosocial care in India? A qualitative case study of three non-profit community mental health organisations. *SSM-Mental Health* 100220.
- Summerfield, Derek, 1999. A critique of seven assumptions behind psychological trauma programmes in war-affected areas. *Soc. Sci. Med.* 48 (10), 1449–1462.
- The Asia Foundation, 2018. *Developing State Sector Counselling Services in Sri Lanka*. Compiled. In: *The Good Practice Guide*. The Asia Foundation, Colombo.
- The Asia Foundation, 2013. Mapping study on the capacity and work experiences of the counseling officers/assistants attached to the ministries of social services and child development and women's affairs. Study Report. The Asia Foundation, Colombo.
- The Asia Foundation, 2016. *Transitions in Mental Health and Psychosocial Support Services in Sri Lanka 2004 – 2015: A Decade in Review*. Colombo: The Asia Foundation.
- Thomas, F.C., Coulombe, S., Girard, T.A., Hart, T.L., Doherty, S., Dass, G., Wickramage, K., Siriwardhana, Ch, Surenthirakumaran, R., McShane, K., 2022. Displacement-related stressors in a Sri Lankan war-affected community: identifying the impact of war exposure and ongoing stressors on trauma symptom severity. *SSM-Mental Health* 2, 100129.
- Thompson, Mindi N., Cole, Odessa D., Nitzarim, Rachel S., 2012. Recognizing social class in the psychotherapy relationship: a grounded theory exploration of low-income clients. *J. Counsel. Psychol.* 59 (2), 208–221.
- Uyangoda, Jayadeva, 2012. Local democracy and citizenship in the social margins. In: Uyangoda, Jayadeva, Mel, Neloufer de (Eds.), *Reframing Democracy: Perspectives on the Culture of Inclusion and Exclusion in Contemporary Sri Lanka*. Social Scientists' Association, Colombo.
- Weaver, L.J., Karasz, A., Muralidhar, K., Jaykrishna, P., Krupp, K., Madhivanan, P., 2023. Will increasing access to mental health treatment close India's mental health gap? *SSM-Mental Health* 3, 100184.
- World Bank, 2024. *Sri Lanka Development Update, April 2024: Bridge to Recovery*. World Bank, Washington, DC. <http://hdl.handle.net/10986/41338>.